

February 19, 2026

Robert Francis Kennedy Jr.
Secretary
Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

Submitted via Regulations.gov

Re: RIN 0955-AA13 Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Dear Secretary Kennedy:

It is with great pleasure that Sanford Health submits our response to your Request For Information “Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care.” Sanford Health believes that the effective and responsible development, implementation and utilization of artificial intelligence technologies will have seismic impacts on the way healthcare is delivered and how patients live their best and fullest lives. By taking a thoughtful and strategic approach to policy development, especially around oversight and approval, reimbursement and privacy, we have the chance to chart a course that will change the health not of just individual patients, but of communities and countries. This is an exciting chapter filled with potential, but fraught with peril if done incorrectly.

Introduction

Sanford Health is the largest rural health system in the United States, serving communities across some of the most geographically expansive and sparsely populated regions in the country. Headquartered in Sioux Falls, South Dakota, Sanford Health is a nonprofit, integrated health system with 55,000 employees caring for more than 2 million patients and nearly 415,000 health plan members across South Dakota, North Dakota, Minnesota, Wyoming, Iowa, Wisconsin, and Michigan’s Upper Peninsula. Our care delivery footprint includes 58 hospitals, 289 clinic locations, 145 senior care communities, and a physician and advanced practice workforce of more than 4,500 clinicians.

Providing high-quality, timely care in rural America presents persistent and well-documented challenges, including long travel distances, workforce shortages, limited specialty access, and chronic underpayment by public payers. For more than a century, Sanford Health has adapted its care models to meet these realities, prioritizing solutions

that preserve local access while connecting patients to the expertise and services they need. Virtual care and digital health innovation have become essential clinical infrastructure for sustaining access across our rural footprint.

Within this context, Sanford Health is thoughtfully integrating artificial intelligence and digital tools to advance three core priorities: access, quality, and sustainability. AI is being applied to address operational and clinical challenges inherent in rural care delivery, including workforce efficiency, care coordination, and timely clinical decision support. Our responses to the questions contained in the initial RFI are below, and are based on our direct experience, reflecting the importance of aligning innovation with real-world care environments, ensuring that emerging technologies are deployed safely, transparently, and in ways that strengthen trust among patients and providers.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Regulatory Uncertainty

The rapid evolution of AI technology and capability make creating a comprehensive regulatory framework difficult, if not impossible. When it comes to regulation, we need a balanced approach that supports innovation, without stifling it, while also protecting patient safety. Clarifying regulations around AI tools and FDA approval requirements on medical devices will alleviate liability concerns for health systems. Also, creating a special reimbursement category for AI-enabled programs is imperative to incentivize provider participation. At Sanford, we have found the implementation of a strong, enterprise-wide AI Governance structure helps to ensure that AI tools are built to internal and external standards and requirements. HHS should prioritize identifying and publicizing best practices in this area so that effective models can be replicated at health organizations of all sizes.

Data Access

If done correctly, AI can turn fragmented data into a connected, community-wide view of health. This would support everything from public health surveillance to personalized care plans. However, data quality and completeness prove to be a barrier. The source and curation of the data affect its completeness, structure, and consistency with coding, leading to a 'gold mine' or a 'dump' of information. There can be multiple EHRs between health systems (sometimes even within health systems) causing fragmented ecosystems. Limited access to data, especially within standardized methodology, is also a barrier. In rural health care systems, there may be limited resources, causing all of the above to be compounded. HHS has a crucial role to play in identifying baseline standards of what type

of data sources are used to train models, how models are updated and refined, and how data can be transmitted across and between systems in an efficient and safe way.

Workflow Integration and Change Management

With technology changing at such a fast pace, there are eager clinicians who are continuous learners and are thriving in this new AI landscape, but for others, the pace of change is challenging. A tremendous amount of change management for health care teams is required not only in the use of AI tools, but also the acceptance of such tools. The ever-changing landscape also requires more technology and IT roles to support our teams and patients. This puts health systems in direct competition with tech-centered companies. To keep up with pace of innovation, it is essential that health systems continually invest in building trust, advancing new infrastructure, continued training and certification and compete for talent in the technology space.

Cost

Creating and purchasing AI tools is very expensive for health systems, and many rural locations simply do not have the funds to fully integrate AI programs and tools to the extent that is needed, in addition to maintaining critical technological upgrades as the industry rapidly changes. In many cases, AI tools are developed to predict chronic diseases. With earlier diagnoses, it may prevent future illnesses and decrease costs long-term. Current payment systems are simply not keeping up with these preventative interventions that best benefit the patient. AI has the potential to fundamentally change how healthcare is delivered. The entities that are able to access new tools and make them part of the normal course of patient care will have an advantage. HHS must ensure that smaller entities that may not have the upfront capital to invest in new systems are not permanently disadvantaged or unable to leverage these tools.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

At Sanford Health, we are focused on accelerating the pace of innovation and adoption of new emerging health care technologies while establishing guardrails to mitigate any potential risks and protect private health information. Any regulation should focus on protecting patients and users of the technology, not becoming too prescriptive or hindering innovation.

Reimbursement and Payment Models

The biggest barriers to the adoption and investment of AI in clinical care is the current structure of reimbursement and payment models, or in reality, the lack of real and meaningful reimbursement models. There is a crucial need to develop novel payment programs to incentivize clinicians and patients to adopt AI tools. Current payment policies, particularly fee-for-service programs, slow AI adoption and/or distort incentives. Creating a special reimbursement category for AI-enabled programs may have large upfront costs but could be very useful to give actors access to emerging technology.

Medicare reimbursement models need to support the use of AI models that not only predict earlier needs for intervention of chronic diseases, but also to support remote monitoring, assisted documentation, and ambient scribing. It is also a necessity that payment models support the integration of AI into clinical workflows without increasing administrative burden. The Medicare and Medicaid programs have the ability to be at the forefront of this work, and providing the technical experience and examples that will also induce greater adoption in the commercial reimbursement markets.

AI Medical Devices

There is much ambiguity around regulations when determining if an AI tool is a decision support tool or may require FDA approval as a medical device. Providing clear guidance that distinguishes FDA-regulated AI/ML medical devices from non-device, clinical-decision support and operational tools will alleviate liability concerns for health systems. Further, FDA should disseminate additional guidance on how AI products will be assessed and regulated once approved, including post-approval surveillance and recertification, as well as the degrees of human involvement are necessary for various technologies. We are not at the place with the technology, from the perspective of both the patients and provider, to fully automate care.

Data Security

It is a fundamental responsibility of health providers to use our utmost care to keep our patients' data safe and private. All providers must operate within secure, resilient, and well-supported digital environments.

At Sanford Health, how we utilize data to improve overall care is something we take very seriously. We have created a multidisciplinary AI/data governance/oversight committee that provides us with a reliable and consistent process to evaluate and determine the efficacy of AI tools (see question #4). The governance framework emphasizes when it is important to keep a human in the loop and our commitment to responsibly develop and

deploy AI models safely and transparently. It is vital to continue to assess how any AI-enabled technology we deploy will use our data and what protections are involved.

HHS already has a robust regulatory framework for data protection in HIPAA. Admittedly, it could be updated to include the plethora of health data that is now sitting outside of current HIPAA standards. For example, there continues to be ambiguity around the use of PHI for developing AI models, requiring more clarification. There is an opportunity to provide safe harbors for de-identification and limited datasets used for AI development.

Flexibility in Clinical Settings

By integrating AI tools across Sanford, we're addressing challenges with targeted solutions, especially in the rural communities we serve. These tools aren't replacing clinicians – they're empowering our care teams to spend more time with patients and make more informed decisions. They're also expanding access, improving affordability, and driving better outcomes for our patients.

By allowing more regulatory flexibility to deploy and evaluate AI tools, health systems would see accelerated adoption and incentivized, effective use of AI tools in the clinical setting. Health systems with the means to create clinical-based AI tools also need more flexibility to expand use of these decision-support tools in order to connect with partners as well as other non-integrated systems. This is especially the case for rural populations, who usually face a more fragmented healthcare delivery experience, and higher rates of chronic condition and acuity of their conditions. We must maintain regulatory flexibility that allows us to build the AI ecosystem that best meets the needs of the patient and communities we serve.

3. What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Data Ownership

There continues to be ambiguity around the ownership and use of health care data to train AI models, creating an increased risk for health systems. Health systems need clarity and flexibility on the rights of this data being used.

Limited Data Sets

Many models are trained in limited data sets which may lead to performance issues or bias when applied to diverse populations. Maintaining HIPPA compliance may also limit model effectiveness.

Cybersecurity Risks

Creating and implementing AI tools may increase cybersecurity risks. As stated previously, it is a fundamental responsibility of health providers to use our utmost care to keep our patients' data safe and private. Reducing cybersecurity risks is extremely expensive and cost prohibitive in many small rural facilities. HHS should create payment models to support these increased costs.

- 4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?**

Evaluation and Validation Methods

Evaluation and validation methods should include using diverse datasets, audits of AI tools across demographic and clinical subgroups, prospective monitoring drift, bias, calibration, and unintended consequences, and continued testing against EHR configuration changes and/or missing data.

Governance Structure

At Sanford Health, we created a multidisciplinary AI/data governance/oversight committee that provides us with a reliable and consistent process to evaluate and determine the efficacy of AI tools. Their function is to evaluate requests from clinicians and other leaders on our team who bring forward ideas as well as outside companies looking to partner with us on new AI tools. We put the patient and the caregiver at the center of every single AI decision.

This governance structure is most focused on ensuring these tools are deployed responsibly, safely, and transparently. Our collaboration with industry partners on a voluntary framework for health care AI commitments is critical to this work.

Creating an independent regulatory board comprised of non-governmental organizations (such as health systems, payers, 3rd party developers, etc.) would provide frameworks and a possible registry for development and use of AI tools. Further, as recommended previously, HHS should prioritize identifying and publicizing best practices in this area so that effective models can be replicated at health organizations of all sizes. The creation of a third-party entity must not create an unnecessary or burdensome level of oversight but instead be fashioned to serve as a catalyst for innovation, adoption and dissemination of best practices in this rapidly expanding space.

Funding Opportunities

There is an opportunity for HHS to fund AI initiatives through grants or more robust AI-supported payment models. Another funding opportunity could be through other cooperative arrangements within HHS, such as supporting AI and innovation research labs or supporting cooperation among health systems.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Accreditation and Certification

Using the independent regulatory board as discussed in question #4 above, the function of the board could expand to include accreditation and certification with active support from HHS. In cooperation with other independent third-party organizations, accreditation programs could be developed to include fairness, transparency, and monitoring standards. Further success could be achieved if HHS encouraged these accreditation bodies to incorporate AI governance into their quality standards. Offering additional support from HHS such as technical guidance aligned with accreditation bodies would also assist with the regulatory board's success as well. It is important to keep in mind that external validators or oversight boards should not create extraneous, redundant or superfluous regulatory bodies that impede the creation and deployment of responsible and effective AI technologies that will benefit patients, healthcare providers, and the well-being of the nation.

Workforce Development

A different skill set is required in today's clinical setting than just a few years ago. In fact, we see that technology is different and/or new by the time someone graduates from a four-year program. The rapid pace of evolving technology is an ongoing barrier within AI adoption. Training an appropriate workforce will be crucial to maximize the impact and potential of this technology.

Sanford Health uses advanced clinical training and simulation to strengthen the rural health care workforce, improve readiness for high-risk clinical scenarios, and expand access to specialized care in communities with limited local resources. By bringing immersive, technology-enabled training directly to rural settings, Sanford equips clinicians across disciplines with the skills, confidence, and support needed to deliver high-quality care close to home. Sanford Health also addresses critical gaps in emergency care through comprehensive EMS education and advanced practice provider (APP) training. Over the past five years, Sanford has enrolled more than 1,500 students in EMT programs and

provides weekly live, interactive continuing education sessions for rural EMS providers. Launched in 2026, the APP Emergency Department Foundational Knowledge and Skills Program delivers more than 150 hours of structured training for nurse practitioners and physician assistants staffing rural emergency departments, with plans to enroll more than 130 APP clinicians over three years. These efforts are further supported by the Virtual Care Center Education Institute, which integrates telehealth and virtual reality into simulation-based training.

Health systems are facing a global battle for talent, especially with roles that can be fully remote. Health systems are competing for talent with every other company that has a technology need across the country – not just Google and Amazon. This barrier can be amplified in rural settings. We will need new roles like data scientists, predictive modelers, AI developers to develop chatbots, product management and people that are focused on the end-to-end patient journey and experience. However, health systems have a unique differentiator in that many people are looking for a career that brings meaning and purpose, and there's no better place to make a difference every day than in health care.

HHS has an opportunity to support workforce development for the many areas and expertise required in the AI setting while elevating the personal fulfillment of working in health care. Overall, more training opportunities and funding for more formal education are key for the future of workforce development.

Since the pipeline to develop talent can be long, HHS should focus on supporting this area without delay. We also need to be cognizant that training should focus on what we need in the future, not where we are at right now. Developing flexible training programs that can adapt with the evolution of technology are imperative.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations, and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Sanford Health has made significant investments in advanced analytics and internally developed AI tools that support both clinical decision-making and operational efficiency. These tools are embedded within the electronic medical record and across care management workflows to help identify patients at risk earlier, prioritize outreach, and support timely intervention. Sanford Health has deployed AI models to identify patients at risk for chronic kidney disease before symptoms appear, flag high-risk obstetric patients for earlier monitoring, identify patients at elevated risk for colon and lung cancer, and predict

the likelihood of developing Type II diabetes prior to diagnosis. Additional AI-enabled tools reduce missed appointments, identify drivers of avoidable emergency department use, and support prevention of post-acute readmissions. Together, these capabilities allow care teams to focus resources where they can have the greatest impact while reducing administrative burden in workforce-constrained rural settings.

To be clear, these tools aren't replacing clinicians – they're empowering our care teams to spend more time with patients and make more informed decisions. They're also expanding access, improving affordability, and driving better outcomes for our patients.

Although AI has not yet met many of the needs for health care, it does continue to advance in many areas such as:

Predicting Chronic Disease States

AI has the potential to reduce gaps in care and identify early warning signs of chronic conditions like diabetes, hypertension, or mental health risks, especially in patients who might not visit frequently. For rural populations, this proactive approach can reduce costly hospitalizations and improve outcomes.

Sanford Health's enterprise data analytics team developed an AI-driven risk score that's now embedded in the electronic medical record. It alerts primary care providers to patients who are at an elevated risk of developing chronic kidney disease. Primary care physicians work closely with nephrologists to define care pathways tailored to each risk category. This ensures patients get the right interventions at the right time. Since implementing the tool, we've seen a significant increase in early diagnoses. Patients identified through the AI model are more likely to follow through with labs, start appropriate medications, and engage in care plans that can potentially delay progression to end-stage renal disease. The tool also helps providers determine when a patient should be referred to a kidney specialist, making follow-up more timely and appropriate. For patients who do progress to dialysis, earlier preparation can mean learning how to manage it at home or making decisions about care location before it's urgent—ultimately improving quality of life and access to care.

With colorectal cancer rates rising and screening eligibility now starting at age 45, Sanford Health built an AI model using 85 risk factors – from genetics to mental health status – to identify patients at higher risk of colon cancer. Those at elevated risk are prioritized for colonoscopy, while average-risk patients can use FDA-approved stool-based tests. The AI model embedded in our EMR will begin rolling out across Sanford in early 2026. While North Dakota has one of the highest colon cancer rates in the U.S., this AI model will help

guide clinicians in their decision-making for recommended screenings – especially critical as we face a shortage of GI doctors in the rural Midwest.

Early intervention with high-risk OB patients can prevent complications like preterm birth or preeclampsia, improving outcomes for both mothers and babies. Our team has developed an AI tool that scans the EMR for clinical indicators to flag expectant mothers at higher risk of complications. Providers are alerted and can initiate earlier, more frequent check-ins.

Rates of lung cancer are rising among non-smokers, particularly women. Traditional screening guidelines often miss these patients. AI can help identify risk earlier and improve screening protocols. Sanford Health is piloting a research-based AI model to identify patients at higher risk for lung cancer using broader data inputs in the electronic medical record – not just smoking history.

Agentic AI supporting staff shortages

We're piloting AI voice-based outreach to help patients schedule appointments and close care gaps (e.g., overdue screenings or follow-ups). The AI-powered platform can tailor communication based on a patient's preferred channel (e.g., phone call, text, letter in the mail, etc.). Personalized outreach helps make preventive care more accessible and catches health issues before they become more serious, improving outcomes.

Ambient Documentation

Sanford Health has also piloted an AI-powered ambient documentation solution to reduce documentation burden and improve efficiency in clinical workflows. The pilot preserved or increased clinician productivity while lowering time spent in the EHR outside working hours and decreasing time in notes. Clinicians engaged in the pilot closed approximately 3,000 additional charts on the same day and 4,500 additional charts within seven days, directly supporting timely follow-up and care coordination. Documentation quality improved with more comprehensive and accurate capture of Hierarchical Condition Categories, enhancing clinical histories, continuity of care, and reimbursement accuracy aligned to patient acuity.

Dictation volume and cost declined markedly: prior to implementation, the cohort averaged 16,000 transcribed lines per month with 70 percent from clinic and ED dictation at about \$2,600 per month; after implementation, transcription fell to 5,000 lines per month, clinic and ED dictation dropped to 6 percent, and costs declined to about \$800 per

month. Enterprise analysis indicates an opportunity to eliminate up to 84 percent of dictation, representing an estimated \$2 million in annual savings.

Additional feedback from providers:

- 100% said they would be disappointed if they couldn't use the tool
- 100% responded that it was easy to use
- 88% reported improvement in burnout or fatigue
- 90% reported higher job satisfaction
- 90% reported better work-life balance
- 95% of clinicians said it reduced cognitive burden
- 80% are more likely to continue practicing

Patient experience remained strong, with favorable provider Net Promoter Scores and “provider spent enough time with me” responses, while clinician surveys indicated reduced documentation burden and higher satisfaction consistent with observed efficiency gains. Eventually, expanding ambient technology to additional clinicians, including nurses and therapists, will increase efficiency across care teams, reduce documentation burden, and expand capacity to scale services.

Operational efficiencies

AI has the power to offset rural workforce shortages by automating administrative tasks, triaging patients and even supporting clinical decision-making.

Sanford Health has built an AI tool that uses patient data to predict patients more likely to miss their appointment. Our teams proactively reach out to address potential barriers – like transportation or work conflicts – and offer earlier appointments when available. Missed appointments delay care and increase health disparities, especially in rural areas where rescheduling can take weeks.

The above examples could also be considered where we are falling short today. Because of HIPPA restrictions and ambiguity and data sharing regulations, this opens the door for tool bias, inconsistencies, and possible errors. Additionally, benefits for utilizing clinical-based AI tools fall to the payors vs. the providers.

Using broader and more standardized data sources, providing funding sources through enhanced payment models, reducing restrictions on HIPPA related data, and having a strong self-governance model in place, would allow us to create much stronger tools that support providers at the bedside and alleviate labor intensive burdens.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Influential Decision Makers and Governing Bodies:

- Chief Medical Officer
- Chief Nursing Officer
- Chief Information Officer
- Chief Digital Officer
- Chief Data/Analytics Officer
- Compliance and Legal Leadership
- Finance Leadership
- Operations Leadership
- Service Line and Hospital administration

Administrative Hurdles for adoption of AI in clinical care include:

With the ever-evolving and fast-paced nature of AI, keeping up to date with its capabilities and potential requires administrative conversations on how to best prioritize its changes. Competing priorities with limited resources leads to difficult decisions and meticulous planning by administrators. Additionally, unless there are more defined guardrails and some clarity in regulation, there will continue to be legal risks of deploying clinical models. Without proper regulation, we may see security risks of deploying clinical models as well. Lastly, although crucial, it will be a hurdle for administrations to develop strong governance policy and procedures in such a diverse and quickly changing AI environment.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Sanford operates a robust set of interoperability tools that support data exchange across hospitals, retail partners, post-acute facilities, pharmacies, and health plans. This infrastructure allows Sanford to deliver coordinated care across geographically dispersed areas without increasing the cost of care.

Despite these capabilities, rural health systems continue to operate within fragmented and state-based interoperability environments. States maintain or propose separate health information exchange structures that create friction for multi-state systems.

Enhanced and expanded interoperability supports integration of wearable devices and remote monitoring into clinical workflows, improving visibility into patient health and enabling new rural care models, including pharmacist-led interventions that improve chronic disease management, medication adherence, utilization, and access.

Enhanced interoperability would expand the market for AI tools to work reliably with reduced bias across diverse populations and settings. Many health care systems that use training models from internal data possibly lack a full longitudinal picture of a population. The risk of a non-diverse population used to train models may create an ineffective and biased clinical tool.

Standardizing clinical notes, imaging, labs, medications, device data, etc. Using FHIR and USCDI, expansion would increase possibilities for research and development of clinical care AI tools. However, this will also require regulatory changes to lessen the legal risks of using expanded data sources.

As interoperability expands, cybersecurity must expand alongside it. Sanford views cybersecurity as a core utility protecting all digital health functions, including AI, telehealth, clinical software, and multistate data exchange.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

What providers want:

- *Time back in their day to focus on patients:* Providers didn't go to medical school to stare at a computer screen. AI can automate routine tasks like documentation, billing codes, prior-authorizations, and help with chart summarization and clinical notes. AI serves as a tool to alleviate administrative burdens.
- *Clinical decision support they can trust:* Evidence-based recommendations are necessary.
- *Seamless integration with EHRs:* If it's not embedded in the workflow, it won't be used. Rural providers are stretched thin — tech must work with their reality, not add complexity.

What patients want:

- *Better access and faster care:* AI-powered chat or triage that shortens wait times and reduces travel. Reminders for screenings, med refills, or follow-ups.
- *Clarity and transparency:* Patients want to know how AI is being used in their care — especially if it influences diagnoses or care decisions.
- *Clear explanations:* Patients are looking for tools to help explain complex medical situations

Both patients and caregivers want better care coordination across multiple providers and are looking for tools to predict chronic and adverse events.

Concerns by patients and/or providers using AI:

Ongoing concerns from both patients and providers are barriers that will take time to overcome, but earning trust in AI tools and showcasing its' potential may alleviate and expedite some of the friction. Concerns from both parties include lack of human connection, privacy and data security, risk of error and not fully understanding AI (fear of the unknown). Providers are concerned about job security and bias/fairness with AI tools. We need to be transparent so that providers and patients understand what AI is doing and why. It's also important we emphasize that AI supports – not replaces – the clinician's judgement. This is especially important in rural areas where the clinician-patient relationship is deeply personal. And lastly, showing evidence and demonstrating how AI improves efficiency, reduces burnout or helps detect health issues earlier. As we continue to integrate AI in the clinical setting, it's vital to gain trust by sharing reputable literature, research and outcomes, choosing high-quality vendors and tools, and highlighting successes in our own clinical settings.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

a. Are there published findings about the impact of adopted AI tools and their use clinical care?

b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

By prioritizing AI research dedicated to the economic effects and impact of payment models, health systems and providers may have more 'buy in' to accelerate adoption of AI in the clinical setting. A stronger trust will be built knowing that the investment of AI is worth the long-term outcomes, as well as knowing that providers will be compensated fairly. Currently, providers are burdened with the up-front costs of developing and purchasing AI tools with payors reaping the benefits. Also, with AI tools relatively new in the clinical setting, many of the cost-benefits are harder to quantify in the short term, including reduced provider burnout.

Research that also showcases the effectiveness and safety of AI tools across diverse populations will build trust and transparency of AI for both providers and patients.

Sincerely,



Brad Reimer, Senior Vice President, Chief Information Officer